Chest CT report

Example report written for this demo. Not a real patient.

Fictional age and history written for this demo.

Example patient O · LIDC-IDRI-0271

Clinical information

44-year-old man. CT pulmonary angiogram. Sudden shortness of breath.

Never smoker. No known cancer.

Technique

Axial CT of the chest with intravenous contrast, 2.5 mm slices.

No prior imaging is available for comparison.

Findings

Chest wall and bones

No acute rib fracture is seen.

The sternum and the visualised thoracic spine are intact.

The soft tissues of the chest wall are unremarkable.

Heart and vessels

The pulmonary arteries are well opacified.

No pulmonary embolism is seen.

The heart is normal in size.

There is no pericardial effusion.

The thoracic aorta is normal in calibre.

Mediastinum and hila

There is no mediastinal or hilar lymphadenopathy.

The oesophagus is unremarkable.

Airways

The trachea and main bronchi are patent.

Lungs and pleura

The lungs are well expanded.

There is no consolidation or pulmonary oedema.

There is no pneumothorax.

There is no pleural effusion.

However, a 7 mm solid nodule is seen in the left mid-lung zone.

No other pulmonary nodule is identified.

Upper abdomen

The visualised upper abdomen is unremarkable.

Impression

1. Pulmonary nodule, as described above.